

DETAILED PRACTICAL GUIDE

# How to support survivors when a specialist is unavailable

UNICEF and IASC partners · English · 2018

**Important:** This is a detailed unofficial reading guide, not an official translation. Consult the original publication for complete requirements and context.

Based on the original publication

[https://www.unicef.org/ukraine/media/22126/file/GBV\\_PocketGuide021718.pdf](https://www.unicef.org/ukraine/media/22126/file/GBV_PocketGuide021718.pdf)

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## 01 Purpose and scope

- A pocket guide for any humanitarian worker or community member who receives a disclosure when no specialised gender-based-violence service is available.

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## 02 Key guidance

- Prepare before a disclosure: know available health, safety, child-protection and emergency services and understand which referrals may be unsafe.
- Introduce your role and limits, find a private setting, and check whether continuing the conversation is safe.
- Listen calmly, believe the person, acknowledge her courage and ask what she needs now; avoid “why” questions and detailed fact-finding.
- Do not search for survivors, investigate, mediate, contact the alleged perpetrator or publicly identify someone as a survivor.
- Share accurate information in a language and format the person can understand, including the right to decline services.
- For children, follow child-safeguarding procedures and involve trained actors; do not make promises of secrecy that conflict with clear protection duties.
- If no specialist exists, identify safe general services and community supports without disclosing the reason for referral unnecessarily.
- After the conversation, store no unnecessary notes, consult supervision without identifying details and care for your own wellbeing.

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## 03 How to use this guide

- This guide turns the source publication into a practical reading aid. It is designed for survivors, trusted supporters, health and social-service staff, and programme managers. It does not replace emergency services, medical care, legal advice, or the complete source publication.
- Use only the parts that fit the person's situation. Preserve her choice, privacy and pace. If a suggested action could expose browsing, calls, documents or location to an abusive person, first choose a safer device, place or time.

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## 04 Survivor-centred principles

- Safety comes first. Ask what would make the next hours or days safer and avoid steps that may provoke retaliation. Never require a disclosure in order to offer basic help.
- Respect autonomy and informed consent. Explain realistic options, benefits, limits and possible risks; the survivor decides what happens next unless an immediate legal safeguarding duty applies.
- Protect confidentiality. Collect the minimum information, restrict access, agree on safe contact methods and explain any limits before information is shared.
- Provide equitable access. Consider disability, language, age, migration status, poverty, rural isolation, sexual orientation and other barriers without stereotyping.

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## 05 Practical response sequence

- Begin with immediate safety and urgent health needs. Ask privately, without the suspected perpetrator present, whether it is safe to talk and whether the person needs emergency care.
- Listen without interrogation. Validate what happened, avoid blame, and do not demand a chronological account or proof. Record only what is necessary and use the survivor's own words.
- Offer a small number of verified options: medical care, safe accommodation, psychosocial support, legal information, police or protection services where appropriate. Explain that accepting one service does not require accepting all others.
- Make a warm referral when consent is given: confirm the service is open, accessible and safe; explain what to expect; and avoid sending the survivor repeatedly between agencies.
- Agree on follow-up and a safe way to communicate. Do not leave identifying messages, send revealing notifications or store files under obvious names if devices may be monitored.

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## 06 Safety and quality checks

- Do not confront the suspected perpetrator, mediate a violent relationship, promise an outcome you cannot control, or pressure a survivor to leave immediately.
- Do not use unverified phone numbers or referrals. Services may close, change eligibility rules or share information in unsafe ways; confirm details before publication or referral.
- Organisations should train staff, supervise difficult cases, create confidential documentation rules, maintain updated referral maps and offer staff support to reduce burnout and harmful practice.

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## Action checklist

- Is it safe to speak, read or keep this document right now?
- Have urgent injuries, pregnancy concerns, sexual-violence care and suicide risk been considered?
- Has the survivor chosen what information may be recorded or shared?
- Are interpretation, disability access, childcare, transport and cost barriers addressed?
- Are referrals verified and is there a safe follow-up plan?
- Has the person been told that the violence is not her fault and that she can decline any option?